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FREE GUIDE

5 Signs Your GLP-1 Is Working Against You

Mounjaro, Wegovy and Ozempic handle your appetite. They don't tell you what to eat — and that gap is where people lose muscle, lose hair, and end up lighter but softer. Here are the five warning signs, and the fix for each one.

40%
of weight lost can be muscle — not fat

<10%
what it drops to with the right protocol

5 fixes
each one takes a week to build in

READ THIS FIRST

The jab manages appetite. Nothing else.

GLP-1 medications are the most effective appetite tools we've ever had. That part isn't in question. But appetite was only ever half the problem — and the half it doesn't touch is the half that decides whether you end up **lean and strong** or just **smaller and softer**.

When you barely feel hunger, you don't just eat less food. You eat less **protein**. You skip meals entirely. You under-fuel training — if training's happening at all. And your body, needing energy from somewhere, takes it from wherever's easiest. Fat, yes. But muscle too. Research on GLP-1 weight loss shows **up to 40% of what's lost can be lean mass** when there's no plan protecting it.

Muscle is the pension fund. It's your strength, your metabolism, your insurance against getting old badly. Every kilo you keep now pays out for decades — and every kilo you lose on the jab is brutally hard to buy back later.

The good news: protecting it isn't complicated. Five signs to watch, five fixes to build in. Each fix takes about a week to become routine. Read the signs honestly — most people on a GLP-1 have at least two.

> *"Losing weight isn't the goal. Losing fat, keeping muscle, and keeping the result — that's the goal."*

01

The scale drops — but you look softer

The scale says minus 8kg and the mirror says... not much. Clothes fit differently but nothing looks **tighter**. That mismatch is the clearest early signal that a chunk of what you're losing is muscle, not fat.

Fat loss changes your shape. Muscle loss just shrinks it. If the shape isn't changing, the composition of your weight loss is wrong — and the scale will never tell you that on its own.

THE FIX — MEASURE MORE THAN YOU WEIGH

Weekly: waist measurement at the navel, same day, same time. **Fortnightly:** front and side photos, same spot, same light. **Daily weigh-ins are fine** — but only the 7-day average counts, never the single number.

If weight is dropping and the waist isn't moving after 2–3 weeks, that's your alarm bell — the fixes on the next pages are now urgent, not optional.

02

Protein has quietly collapsed

Appetite suppression makes protein the first casualty. A slice of toast goes down easier than a chicken breast when nothing appeals — and most people on a GLP-1 are eating **half** the protein they need without realising.

Here's the part that surprises everyone: on a GLP-1 you need **more** protein than a normal dieter, not less. The bigger the deficit and the lower the appetite, the harder your body looks at muscle as a fuel source.

1.2–1.6g	20–40g	First
per kg of bodyweight, per day	per meal — your body uses it best in doses	protein goes on the plate before anything else

THE FIX — HIT THE NUMBER WITH NO APPETITE

Protein first, always: when appetite is small, spend it on the thing that matters.
Make it easy: Greek yoghurt, eggs, cooked chicken in the fridge, a shake counts.
Anchor it to meals you already have — 30g at breakfast is worth more than a perfect plan you won't follow.

Quick check: bodyweight in kg × 1.4. That's your daily target. Write it down now.

03

Energy's gone by mid-afternoon

Under-fuelling feels like discipline right up until it feels like exhaustion. Dead by 3pm, training flat, snapping at people, sleep getting worse — that's not willpower working, that's your body running on fumes.

The jab makes it dangerously easy to eat far below what your body can sustain. A deficit drives fat loss. A **crater** drives muscle loss, rebound hunger, and the kind of fatigue that makes you quit the whole thing by month three.

THE FIX — A DEFICIT, NOT A CRATER

Eat by the clock, not by hunger: three anchored meals, even when small. Hunger is no longer a reliable signal — structure replaces it. **Carbs around activity:** you don't need them gone, you need them placed. **If energy's still flat after two weeks** of proper protein and regular meals, your total intake is too low — nudge it up, the fat loss won't stop.

> *"The plan you can live on beats the plan that impresses people. Every time."*

04

Hair shedding, nails weak, training flat

More hair in the plughole. Nails that chip. Lifts going backwards, or walks that used to be easy feeling like effort. These are classic signs that the weight coming off includes **tissue you wanted to keep** — your body triaging a shortage of protein and nutrients by cutting the non-essentials.

It's fixable. But it only gets fixed if you catch it and respond — it does not sort itself out while the deficit continues.

THE FIX — GIVE YOUR BODY A REASON TO KEEP MUSCLE

Lift 2–4 times a week. This is the single strongest signal you can send: muscle that gets used doesn't get burned. Full-body, 30–45 minutes, basic movements done well — nobody needs a bodybuilder's programme. **Pair it with the protein target from Sign 02** and the research says muscle loss drops from as much as 40% to under 10%.

Not trained in years? Start with two sessions. The habit matters more than the programme for the first month.

05

No plan for when you come off it

The most expensive sign of all, because it shows up last. The jab manages appetite — it doesn't build habits. Come off it with nothing underneath and the appetite returns to a person who hasn't changed how they shop, cook, eat or train. The research is blunt about what happens next: **most of the weight comes back**.

The people who keep their results treat the medication as scaffolding: it holds things steady while the real structure gets built. The ones who regain treated it as the building.

THE FIX — BUILD THE SYSTEM WHILE THE JAB BUYS YOU TIME

While appetite is quiet is the easiest time you will ever have to build the habits: the protein-first plate, the 2–4 sessions, the weekly weigh-and-measure routine, a shopping list that makes good choices automatic.

The test: could you describe your eating system in three sentences without mentioning the jab? If not, that's the work — and it's exactly the work I do with clients every week.

BONUS

The Control List — trolley rules

You don't need a perfect diet. You need a trolley that makes the good choice the easy choice. These are the rules I give every client:

- **Protein anchors the shop:** chicken, mince (5% fat), eggs, Greek yoghurt, fish, cottage cheese — every meal gets built around one of these.
- **Volume foods fill the gaps:** potatoes, rice, oats, frozen veg, salad, fruit — food that fills you without costing much.
- **One-step foods win:** if it can be eaten with zero cooking (yoghurt, cooked meats, fruit), it survives a low-appetite day. Stock accordingly.
- **Never shop hungry — or on a jab-day appetite crash.** You buy with your eyes either way; make sure your eyes are calibrated.
- **Eating out (Saturday rules):** protein first — pick the dish around the meat or fish. One indulgence, not three. Never arrive starving.

> *"A meal out never ruined a diet. The 'sod it, week's ruined' mindset after it did."*

WHAT NOW

Two ways forward

Do it yourself: everything in this guide works on its own. Set the protein number, book the two lifting sessions in your diary, do the weekly measure. Start with the sign that stung the most when you read it.

Or do it with the numbers watched for you: I coach men 35–50 — personalised macros, training you can recover from, and a proper review of your weight trend, photos and training every single week. GLP-1 nutrition support is my specialty: keeping the muscle while the scale drops, and building the system that holds when you come off it.

Fancy a straight conversation about whether it's a fit?

No pitch, no pressure — if it's not right for you, I'll say so.

Email callum@callumbcoaching.co.uk — or message me on Instagram @callum.transformation.coach

This guide is general nutrition and training information, not medical advice. Never change your medication or dosing without speaking to your prescriber. If you're experiencing significant side effects, talk to your GP.